

1. NSAIDs

WHAT YOU SEE

NSAID bottle on the acute-treatment shelf

WHAT IT ENCODES

Acute flare first-line in patients without contraindications: full-dose NSAIDs (e.g., indomethacin 50 mg TID, naproxen 500 mg BID) continued until 1–2 days after symptoms resolve, typically 5–7 days. Earlier initiation (within 24h) means faster control. Any NSAID works — choose by tolerability, not potency.

BOARD TRAP

WRONG answer: prescribing an NSAID to a flare patient with CKD, active peptic ulcer, heart failure, or on anticoagulation. NSAIDs cause AKI, GI bleeding, and fluid retention. Discriminator: in CKD/HF/PUD/anticoagulated patients, switch to oral or intra-articular glucocorticoids (or IL-1 inhibitor) — not NSAIDs or full-dose colchicine.

2. Colchicine 1.2/0.6

WHAT YOU SEE

Bottles labeled 1.2 and 0.6 with a 1-hr clock

WHAT IT ENCODES

Colchicine inhibits microtubule polymerization, blocking neutrophil chemotaxis and NLRP3 inflammasome activation. Acute-flare dosing is LOW-DOSE: 1.2 mg at onset, then 0.6 mg one hour later (1.8 mg total day 1), far better tolerated than the old high-dose 'until diarrhea' regimen. Most effective when started within 24 hours of flare onset.

BOARD TRAP

WRONG answer: giving standard-dose colchicine with a strong CYP3A4 or P-glycoprotein inhibitor (clarithromycin, ketoconazole, cyclosporine, verapamil, diltiazem) or in significant renal/hepatic impairment. This causes life-threatening colchicine toxicity. Discriminator: clarithromycin + colchicine is a classic lethal combo — reduce/hold colchicine; azithromycin is safer.

3. Glucocorticoids

WHAT YOU SEE

Steroid/GC syringe and PRED bottle on the shelf

WHAT IT ENCODES

Glucocorticoids are first-line when NSAIDs and colchicine are contraindicated. Options: oral prednisone ~0.5 mg/kg/day for 5–10 days with taper; intra-articular triamcinolone for mono/oligoarticular flares; IM/IV or ACTH if NPO. Equally effective to NSAIDs for acute gout. Preferred agent in CKD and in the elderly.

BOARD TRAP

WRONG answer: injecting a joint with steroid (or labeling it gout) before excluding infection. Discriminator: never give intra-articular glucocorticoid until septic arthritis is ruled out by aspiration (Gram stain/culture) — steroid into a septic joint is catastrophic.

4. Skull = colchicine toxicity

WHAT YOU SEE

Skull-and-crossbones above the shelf

WHAT IT ENCODES

Colchicine has a narrow therapeutic index. Toxicity progresses GI first (nausea, vomiting, profuse diarrhea), then multi-organ: bone marrow suppression (pancytopenia), neuromyopathy (proximal weakness, elevated CK), and in overdose multi-organ failure. Risk rises with renal/hepatic impairment and CYP3A4/P-gp inhibitors.

BOARD TRAP

WRONG answer: managing colchicine overdose with hemodialysis. Colchicine is highly tissue-bound with a large volume of distribution, so dialysis does NOT remove it. Discriminator: care is supportive (the colchicine-specific Fab antibody is not generally available); prevention via dose limits is key.

5. DO NOT MIX acute+chronic

WHAT YOU SEE

Chained sign 'DO NOT MIX' over fire (ACUTE) and crystals (CHRONIC)

WHAT IT ENCODES

Do NOT START urate-lowering therapy during an acute flare — abrupt urate shifts mobilize crystals and prolong/worsen the attack. Conversely, if a patient is ALREADY on stable ULT, do NOT STOP it during a flare; treat the flare on top of it. Start ULT ~2 weeks after the flare settles (or, per newer ACR guidance, may start during a flare only if anti-inflammatory prophylaxis is co-administered).

BOARD TRAP

WRONG answer: discontinuing the patient's allopurinol because they presented with an acute flare. Discriminator: stopping established ULT causes urate to swing and aggravates the flare — continue it and add NSAID/colchicine/steroid for the acute attack.

6. HLA-B*5801

WHAT YOU SEE

Big banner 'B*5801' with silhouettes

WHAT IT ENCODES

HLA-B*5801 strongly predicts allopurinol hypersensitivity syndrome (SJS/TEN, DRESS). Screen BEFORE starting allopurinol in high-prevalence groups: Han Chinese, Thai, Korean (with CKD), and African ancestry. A positive test means use an alternative (febuxostat) instead.

BOARD TRAP

WRONG answer: switching a B*5801-positive patient to a different antigout drug that is also a xanthine oxidase inhibitor and assuming it's risky too. Discriminator: febuxostat does NOT cross-react with HLA-B*5801 and needs no screening — it is the appropriate alternative when allopurinol is contraindicated by genotype.

7. Allopurinol knight

WHAT YOU SEE

Knight with sword labeled ALLO fighting a urate imp, green check 'B*5801 OK'

WHAT IT ENCODES

Allopurinol is a xanthine oxidase inhibitor and the FIRST-LINE urate-lowering agent (also cheap, once-daily, safe in stones/overproducers). Start LOW — 100 mg/day, or 50 mg/day in CKD — and titrate every 2–4 weeks to a serum urate <6 mg/dL (max ~800 mg/day). Low-start reduces both early flares and hypersensitivity.

BOARD TRAP

WRONG answer: continuing full-dose allopurinol in a patient on azathioprine/6-mercaptopurine. Allopurinol blocks xanthine oxidase, which normally degrades these thiopurines, causing dangerous accumulation and profound myelosuppression. Discriminator: avoid the combo, or reduce thiopurine to ~25% with close CBC monitoring; consider switching the immunosuppressant.

8. Start low / titrate

WHAT YOU SEE

Tablet '100mg' near the king/throne

WHAT IT ENCODES

The mantra is 'start low, go slow, treat to target.' Dose by serum urate goal (<6 mg/dL; <5 for tophi) — NOT by symptom resolution. Recheck urate every 2–4 weeks during titration. Co-prescribe anti-inflammatory prophylaxis during the up-titration window.

BOARD TRAP

WRONG answer: stopping up-titration once the patient is asymptomatic even though urate is still >6. Discriminator: gout is managed treat-to-target; sub-target urate leaves crystal load behind and recurrent flares — keep titrating to <6 mg/dL regardless of symptom status.

9. Febuxostat king

WHAT YOU SEE

Crowned king on throne (the 'royal' second-line agent)

WHAT IT ENCODES

Febuxostat is a non-purine xanthine oxidase inhibitor used when allopurinol fails, is not tolerated, or is contraindicated (e.g., HLA-B*5801 positive). No HLA screening required and usable in mild–moderate CKD without major dose change.

BOARD TRAP

WRONG answer: choosing febuxostat first-line in a patient with established cardiovascular disease. The CARES trial led to an FDA boxed warning for increased cardiovascular and all-cause mortality vs allopurinol. Discriminator: prefer allopurinol first-line; reserve febuxostat for allopurinol-intolerant/contraindicated patients, and use caution in CVD.

10. Probenecid / kidney stone

WHAT YOU SEE

Scroll 'STARTING ULT' with 'KIDNEY STONE' note

WHAT IT ENCODES

Probenecid is a uricosuric that blocks URAT1, increasing renal urate excretion. It is an add-on/alternative when xanthine oxidase inhibitors are insufficient or not tolerated, and works best in documented under-excretors with good renal function. Advise high fluid intake +/- urine alkalinization to reduce stone risk.

BOARD TRAP

WRONG answer: starting probenecid in a patient with urate kidney stones, urate overproduction (24-hr urine urate >800 mg), or CKD (GFR <30–50). It dumps urate into urine and precipitates stones, and is ineffective in renal impairment. Discriminator: in stone-formers/overproducers use a xanthine oxidase inhibitor (allopurinol), not a uricosuric.

11. Flare prophylaxis

WHAT YOU SEE

Targets/coins '≥2/yr' near the start-ULT scroll

WHAT IT ENCODES

When initiating ANY urate-lowering therapy, give anti-inflammatory prophylaxis — low-dose colchicine 0.6 mg daily/BID (or low-dose NSAID, or low-dose prednisone if both contraindicated) — for at least 3–6 months to prevent the mobilization flares triggered by falling urate.

BOARD TRAP

WRONG answer: skipping prophylaxis because the patient feels well at ULT start. Discriminator: early flares are most common in the first months of ULT and are the leading cause of patients abandoning therapy — always co-prescribe prophylaxis when starting/up-titrating ULT.

12. ULT indications

WHAT YOU SEE

'STARTING ULT ≥2/yr, KIDNEY STONE' scroll

WHAT IT ENCODES

Indications to START urate-lowering therapy: ≥2 flares per year, any tophi, radiographic joint damage from gout, urate nephrolithiasis, or CKD stage ≥3 with gout. (Some guidelines also consider it after a single flare with very high urate or young age.)

BOARD TRAP

WRONG answer: starting lifelong ULT after a first, single, uncomplicated flare in an otherwise healthy patient. Discriminator: a one-off flare without tophi/stones/frequent recurrence does not by itself mandate ULT — and asymptomatic hyperuricemia is never an indication.

13. Rule out / 3-6 mo

WHAT YOU SEE

Clinicians labeled 'RULE OUT' and '3-6 mo'

WHAT IT ENCODES

Optimize reversible/secondary contributors when starting ULT: stop or swap urate-raising drugs where possible (thiazide/loop diuretics → consider losartan or amlodipine which are mildly uricosuric/urate-neutral; low-fat dairy and weight loss help), and continue flare prophylaxis for 3–6 months. Always exclude septic arthritis before attributing a hot joint to gout.

BOARD TRAP

WRONG answer: abruptly stopping a needed thiazide solely to treat gout. Discriminator: substitute rather than simply discontinue (e.g., losartan has a mild uricosuric effect, fenofibrate also lowers urate) and weigh the original indication (HTN/edema) — don't compromise BP control for a modest urate effect.

14. Pegloticase cauldron

WHAT YOU SEE

Brewer at cauldron labeled REFRACTORY / PEG

WHAT IT ENCODES

Pegloticase is pegylated recombinant uricase given IV every 2 weeks for severe REFRACTORY tophaceous gout when oral ULT has failed; it rapidly dissolves tophi by converting urate to soluble allantoin.

BOARD TRAP

WRONG answer: continuing pegloticase when pre-infusion serum urate rises back above 6 mg/dL, or giving it in G6PD deficiency. A rising urate signals anti-drug antibodies and predicts severe infusion/anaphylactic reactions. Discriminator: monitor pre-dose urate and STOP if it climbs; screen and avoid in G6PD deficiency (hemolysis/methemoglobinemia from H2O2).